NuVasive Clinical Services

10275 Little Patuxent Parkway Suite 300
Columbia, MD 21044
phone. 410.740.2375 | **fax.** 410.740.1518

Intraoperative neurophysiology report

Procedure details

Patient: Adolfo Torres

Date of birth: 06/27/1961

Report date: 11/25/2025

Service date: 11/24/2025

Case number: 2088577-1661157

Hospital: THE HOSPITALS OF PROV - EAST (1972709970)

Surgeon: Fadi Hanbali, MD (1629175914)

MR number: 518486881

Neurophysiologist: Lauren Gutierrez

Neurophysiologist hours: 12:32 PM - 2:47 PM (2hr 15min)

Reading physician(s): Peien Emily Fan, MD

Monitoring duration: 1hr 6min

Procedure: Spine: Minimally invasive Instrumentation/Fusion

Spinal level: Starting level l4, ending level l5

Diagnosis: [M54.16] [M48.062]

Monitored modalities

Type	CPT code	Unit(s)
Needle EMG - 2 Limbs	95861	1
SSEP - Upper and Lower limb	95938	1

Electromyography was recorded via needles placed in the following muscles:

Note: *B=bilateral; L=left side only; R=right side only*

Category	Muscle
Legs	Biceps Femoris (short head) (B)
Legs	Medial Gastrocnemius (B)
Legs	Tibialis Anterior (B)
Legs	Vastus Lateralis (B)
Legs	Vastus Medialis (B)

Findings and analysis

This case was remotely monitored in real-time.
Modalities:
Upper and lower extremity somatosensory evoked potentials (SEP) were obtained by stimulating ulnar nerve and posterior tibial nerve (PTN) bilaterally.

Free run electromyography (EMG) was recorded from vastus lateralis (VL), vastus medialis (VM), biceps femoris (BF), tibialis anterior (TA), medial gastrocnemius (MG) bilaterally

Baseline bilateral upper and lower extremity SEP were within normal limits with comparatively smaller amplitudes in bilateral lower extremity responses. SEP responses varied in accordance with hemodynamic, thermal and anesthetic alterations. No significant changes were observed as the case progressed. At closing bilateral upper and lower extremity SEP were within baseline parameters. There was no evidence of neuro-electrophysiological injury during surgery based on the findings.

Baseline EMG with use of needle electrodes was monitorable and quiet at baseline. Transient EMG as seen was relayed to the surgeon. No sustained EMG activity was seen. Please refer to log for detailed interpretation on individual muscle groups. Closing EMG was quiet.

The surgeon was updated on the data and its reliability throughout the recordings.

Electronically signed by:
Peien Emily Fan, MD
Approved: 11/24/2025 4:31 PM PST